

ANALYSTLAB AFRICA
EXPERIENCE LAB INTERNSHIP PROGRAMME

HEALTHCONNECT CLINIC EXPERIENCE LAB

PROJECT SCOPE STATEMENT

Improving Patient Appointment Attendance and Healthcare Support Using Data and AI

Document Control

Project	HealthConnect Clinic Experience Lab
Programme	AnalystLab Africa Experience Lab
Document Type	Project Scope Statement
Version	1.0
Project Phase	Initiation
Issue Date	August 2026
Status	Baseline for Project Planning

1. PURPOSE OF THE SCOPE STATEMENT

This Scope Statement defines the boundaries of the HealthConnect project, including the work required to achieve the intended project outcomes, the principal deliverables, exclusions, constraints and conditions governing scope.

It provides the basis for controlling project work and evaluating proposed changes throughout delivery.

2. PROJECT SCOPE

The HealthConnect project covers the assessment, design, development and validation of data- and AI-enabled approaches that can support:

- Improved understanding of appointment non-attendance
- More proactive patient engagement
- Improved access to routine appointment information
- More efficient handling of repetitive patient enquiries
- Better utilisation of appointment capacity
- Data-informed operational decision-making

IN SCOPE: Extends from problem and data understanding through solution validation and implementation readiness.

OUT OF SCOPE: Does not extend to autonomous clinical decision-making or unrestricted production deployment.

3. SCOPE COMPONENTS

The project scope is organised around six major components.

3.1 Data & Problem Understanding

The project will establish an evidence base by:

- Reviewing available appointment and patient-related data
- Assessing data quality, completeness and suitability
- Identifying relevant attendance patterns
- Defining key analytical questions
- Establishing baseline measures where data permits

3.2 Predictive Analytics

The project will evaluate whether machine learning can support identification of appointments or patient circumstances associated with higher likelihood of non-attendance. Activities may include:

- Feature identification
- Data preparation
- Model development
- Model evaluation
- Performance comparison
- Interpretation of relevant results

3.3 Patient Engagement

The project will explore interventions that can support appointment attendance, including:

- Reminder strategies
- Proactive engagement
- Intervention timing
- Appropriate communication channels
- Escalation requirements

The intervention design will be informed by available evidence rather than assuming that prediction alone will reduce no-shows.

3.4 Generative AI Patient Support

The project will explore a Generative AI-enabled support capability for routine appointment-related enquiries. Potential functions include:

- Frequently asked question support
- Appointment procedure guidance
- Basic administrative information
- Conversational support
- Escalation to human staff where required

The capability will operate within defined information and escalation boundaries.

3.5 Process & Workflow Integration

Proposed analytical and AI capabilities will be mapped to relevant clinic processes. This includes:

- Identifying where outputs enter existing workflows
- Defining responsible users
- Establishing handoffs
- Identifying escalation points
- Assessing operational feasibility
- Identifying process changes required for adoption

3.6 Testing, Evaluation & Readiness

The project will assess whether proposed solutions are:

- Technically functional
- Sufficiently reliable for their intended purpose
- Usable by intended users
- Operationally feasible
- Appropriately governed
- Suitable for further implementation consideration

4. MAJOR PROJECT DELIVERABLES

The project is expected to produce the following major deliverables:

Deliverable	Purpose
Problem & Requirements Definition	Establish the operational and patient-support requirements
Data Assessment	Determine data availability, quality and suitability
Attendance Analysis	Identify patterns and factors associated with non-attendance

Deliverable	Purpose
Predictive Model / Approach	Assess the feasibility of predicting appointment non-attendance
Patient Engagement Strategy	Define appropriate intervention opportunities
Generative AI Support Concept	Provide structured support for routine enquiries
Integrated Workflow	Connect solution outputs with clinic processes
Testing & Validation Results	Assess technical and operational performance
Implementation Readiness Assessment	Identify requirements for potential adoption
Monitoring & Evaluation Framework	Define how outcomes can be tracked
Final Recommendations	Consolidate findings and implementation considerations

5. FUNCTIONAL SCOPE

The proposed solution should support the following functional capabilities:

Capability Area	Scope
Appointment Intelligence	- Analyse appointment attendance patterns - Identify relevant predictors of non-attendance - Generate interpretable risk or prioritisation outputs where appropriate
Patient Engagement	- Support targeted appointment reminders or interventions - Enable proactive engagement based on defined criteria - Support escalation where intervention requires human involvement
Patient Support	- Respond to defined routine appointment-related enquiries - Provide consistent information - Identify interactions requiring escalation
Operational Support	- Present relevant insights to appropriate users - Support informed scheduling and engagement decisions - Provide information that can contribute to operational monitoring

6. NON-FUNCTIONAL REQUIREMENTS

The solution should be assessed against the following requirements:

Requirement	Scope Expectation
Reliability	Outputs should be sufficiently consistent for their intended use

Requirement	Scope Expectation
Usability	Intended users should be able to understand and use relevant outputs
Explainability	Important analytical outputs should be interpretable
Safety	AI-supported interactions should operate within defined boundaries
Privacy	Patient information should be handled according to applicable requirements
Scalability	The proposed approach should allow consideration of future expansion
Maintainability	Models, knowledge sources and workflows should be capable of appropriate review and updating
Human Oversight	Appropriate decisions and escalations should remain subject to human control

7. OUT OF SCOPE

The following activities are explicitly excluded from the initial project scope:

EXCLUDED FROM INITIAL PROJECT SCOPE

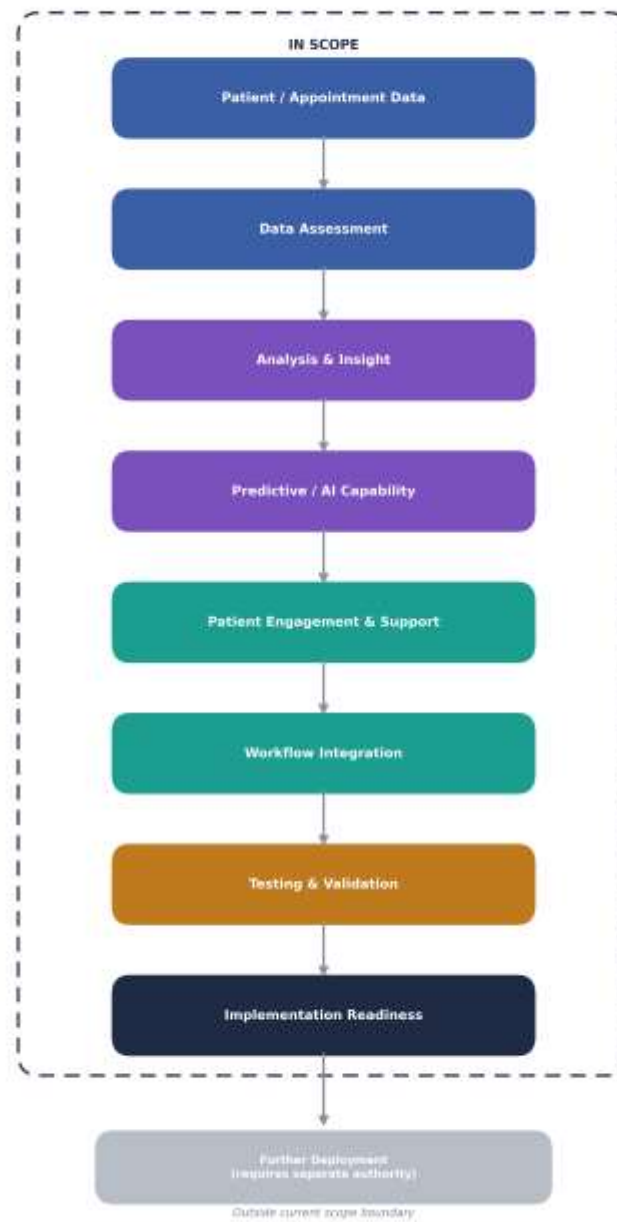
- ✗ Clinical diagnosis
- ✗ Treatment recommendations
- ✗ Autonomous clinical decision-making
- ✗ Replacement of healthcare professionals
- ✗ Emergency medical support
- ✗ Development of a complete Electronic Health Record system
- ✗ Full hospital management software
- ✗ Unrestricted patient-data collection
- ✗ Uncontrolled automated patient communication
- ✗ Production deployment without appropriate approval and validation
- ✗ Healthcare use cases unrelated to appointment attendance and patient support

8. SCOPE BOUNDARIES

The project boundary can be represented as follows. The project boundary ends at implementation readiness unless further deployment authority is formally established.

PROJECT SCOPE BOUNDARY

The project boundary ends at implementation readiness.



9. SCOPE ASSUMPTIONS

The following assumptions apply to the current scope baseline:

1. Relevant data will be available for analysis
2. Available data will be sufficiently representative for the intended analytical objectives
3. Required stakeholders will participate in validation activities
4. Technical resources required for development will be available
5. Proposed AI capabilities can be evaluated within the project environment
6. Relevant privacy, security and responsible-AI requirements can be incorporated
7. The project will have access to sufficient information about relevant clinic workflows

10. SCOPE CONSTRAINTS

Scope may be constrained by:

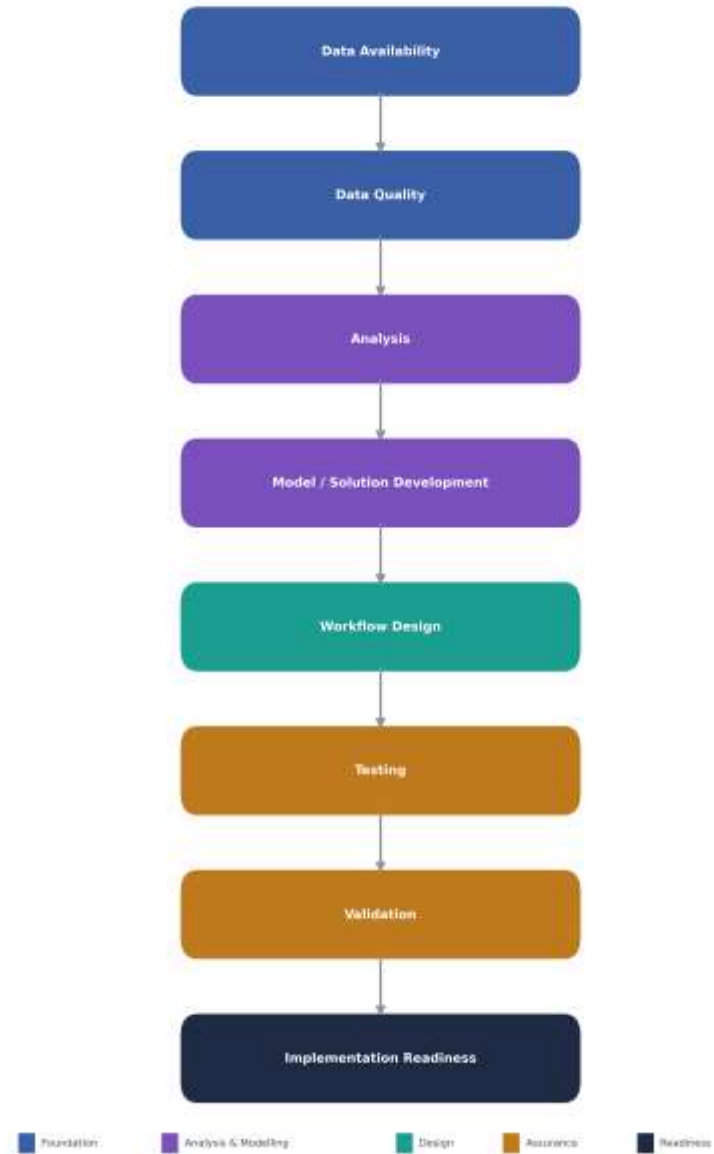
- Data availability and quality
- Simulated or limited healthcare data
- Project duration
- Technical resources
- Access to real-world healthcare workflows
- Availability of subject-matter expertise
- Validation limitations
- Privacy and responsible-AI requirements
- Stakeholder availability

11. SCOPE DEPENDENCIES

Successful delivery depends on the following dependency chain. Failure or delay at an upstream stage may affect downstream deliverables.

SCOPE DELIVERY DEPENDENCIES

Failure or delay at an upstream stage may affect downstream deliverables



12. SCOPE CONTROL

Any proposed change that materially affects the agreed scope should be assessed against:

- Business value
- Patient value
- Project objectives
- Schedule impact
- Resource requirements
- Technical feasibility
- Risk exposure
- Dependencies
- Expected outcome

Changes that materially alter the approved project boundary should require formal review and approval before incorporation into the project baseline.

13. SCOPE ACCEPTANCE CRITERIA

The project scope will be considered adequately addressed when:

1. The appointment attendance problem has been clearly defined
2. Relevant data requirements have been established
3. Attendance patterns and relevant factors have been analysed
4. Appropriate predictive or analytical approaches have been evaluated
5. Patient engagement opportunities have been defined
6. The Generative AI support use case has been appropriately bounded
7. Proposed capabilities have been mapped to operational workflows
8. Key risks, limitations and dependencies have been assessed
9. Proposed solutions have undergone appropriate validation
10. Implementation requirements and monitoring considerations have been documented

14. SCOPE BASELINE

This Scope Statement establishes the initial project boundary for HealthConnect.

The scope may be refined as new evidence, requirements, technical findings or stakeholder decisions emerge. Any material change will be evaluated through the project's change-control process.

HealthConnect will investigate, design and validate data- and AI-enabled approaches for improving appointment attendance and routine patient support, extending from problem and data understanding through implementation readiness.

Document Status

Document	HealthConnect Project Scope Statement
Version	1.0
Project Phase	Initiation
Status	Baseline for Project Planning
Project Management Owner	Project Management Function
Review Authority	Project Sponsor / Project Leadership
Next Review	Following requirements and solution analysis

End of Project Scope Statement